

Pilot Proposal

Sarawak MedChain × Sarawak General Hospital | 4 weeks | Zero cost to SGH

Prepared by	Randy Richard, Founder, MedChain Enterprise
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Status	Draft v1 - to be refined after meeting with Dr Steve and SGH stakeholders
Cost to SGH	RM 0

Objective

Validate that patient-controlled medical records work in a real clinical setting at SGH, measuring doctor adoption, patient comprehension, and technical reliability over a 4-week period.

This is a **parallel pilot** — it does not replace or interfere with SGH's existing record system. Clinical decisions during the pilot continue to rely on SGH's primary system.

Scope

Dimension	v1 Pilot
Duration	4 weeks
Doctors	3, from a single department (suggested: General Medicine or any clinic Dr Steve oversees)
Patients	20 consenting outpatients
Records	Outpatient consultation notes (text + simple attachments)
Site	One SGH clinic, single specialty
Cost to SGH	RM 0 - all infrastructure covered by MedChain Enterprise

What SGH Provides

- 3 doctor accounts (we verify on-chain via the admin role)
- 20 willing patients recruited with informed consent
- One small meeting room for ~2 hours of patient onboarding
- Departmental sign-off from Dr Steve (or designated supervisor)
- 1 nominated SGH IT contact for technical liaison (~1 hour/week)

What MedChain Enterprise Provides

- The full platform — smart contract, backend, frontend
- One 90-minute training session for the 3 doctors
- On-site patient onboarding (MetaMask wallet setup + walkthrough) — Randy on-premises Day 1
- Ongoing technical support throughout the 4 weeks (WhatsApp + email)
- Weekly status reports to Dr Steve
- Final report (clinical, technical, UX findings) at end of pilot

bullet All cloud, IPFS pinning, and RPC infrastructure costs

Success Metrics

Metric	Target
Records uploaded per consulted patient	>= 80%
Platform uptime during clinic hours	>= 99%
Patient access-grant events logged	>= 50 across pilot
Doctor satisfaction (NPS, end of pilot)	>= 7/10
Unauthorised access incidents	0

Out of Scope (deliberately deferred to v2)

- bullet Integration with SGH's existing HIS/EMR (v1 is parallel, manual entry)
- bullet Inpatient or emergency department workflows
- bullet Imaging / DICOM / lab results
- bullet Insurance claim integration
- bullet Multi-hospital network features

We're deliberately keeping v1 small so the team can finish it, learn from it, and present a real result — not get stuck negotiating a big-bang integration.

Risk & Safety

- bullet **Parallel system, not replacement.** SGH's existing records remain the source of clinical truth during pilot. MedChain entries are a copy for evaluation.
- bullet **Informed consent.** Each pilot patient signs a consent form (joint MedChain + SGH MAC review) before being onboarded.
- bullet **PDPA 2010 compliance.** All patient data stored on infrastructure located in Malaysia. Encryption keys are held by the patient (via MetaMask), not by us.
- bullet **Withdrawal at any time.** Any patient or doctor can withdraw without explanation. Their data is revoked on-chain immediately.
- bullet **Open-source contract.** The smart contract source is published — SGH IT can audit it before pilot start.

Timeline (after SGH approval)

Week	Activity
Week -2	Joint sign-off on consent form. SGH MAC review (if required).
Week -1	Patient recruitment by SGH. Doctor onboarding (90-min session).
Week 1	Pilot start. Randy on-site Day 1 for patient onboarding. Daily check-ins.
Week 2	First weekly report to Dr Steve. Mid-week support call.
Week 3	Mid-pilot review meeting. Adjustments if needed.
Week 4	Final week. Doctor + patient NPS surveys collected.
Week +1	Final report delivered to Dr Steve and SDEC.

What Happens After the Pilot

The joint outcome — pilot report — is delivered to **Dr Steve, SGH leadership, and SDEC**. It informs whether and how to design a Phase 2:

- bullet larger department, more doctors and patients?
- bullet HIS/EMR integration as a real project?
- bullet Cross-hospital portability (Miri ↔ Kuching ↔ Sibul)?

Phase 2 is a separate conversation with separate sign-off. No commitment is implied by participating in the pilot.

Single-Page Summary

SGH lends us **3 doctors + 20 patients + 1 small clinic** for 4 weeks.

We bring **the platform, the training, and the support — at no cost**.

At the end, everyone gets **a clean report on what worked and what didn't**.

If it didn't work, we walk away clean. If it did, SDEC and SGH have a real data point to plan Phase 2 from.

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